

D912

Healthcare Quality

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A hospital recently featured in a regional magazine tracks the percentage of surgical patients who get antibiotics within 60 minutes before incision. Its surgical wing has 12 operating rooms. In Donabedian's model, this is which type of measure?

- A. Outcome measure, since any percentage is an outcome
- B. Process measure
- C. Structure measure, because the wing has 12 operating rooms
- D. Outcome measure, because timely antibiotics prevent infections

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

For the same surgical program, which of these is an outcome measure?

- A. The 30-day surgical site infection rate
- B. The number of board-certified surgeons on staff
- C. The percentage of patients given timely antibiotics
- D. Any measure that is publicly reported

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A clinic in a town with a new minor-league stadium wants to cut no-shows. Staff predict that text reminders sent two days ahead will lower no-shows, and they try it with one provider for two weeks. The clinic manager just finished her MBA. What happens in the Study step?

- A. Replace the test with a year-long clinic-wide study
- B. Send the texts and record no-shows
- C. Roll the reminders out to every provider
- D. Compare the no-show results with the prediction and note what was learned

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A lab turnaround-time control chart at a hospital that recently renovated its lab shows daily values varying within the control limits with no pattern. On Tuesday, one value is higher than Monday's but still well within the limits. The lab director is a former Army medic. What should the team do?

- A. Investigate Tuesday as a special-cause event
- B. Treat it as common-cause variation and not react to the single point
- C. Recalculate the control limits after each new point
- D. Investigate immediately, since any rise always needs action

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

On a control chart, which pattern signals special-cause variation?

- A. Any single point above the center line
- B. A point higher than the previous one but within limits
- C. All points scattered randomly within the limits
- D. A single point outside the upper control limit

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A medication-error review at a hospital with a new pharmacy director found 100 errors: wrong dose 45, wrong time 25, omitted dose 15, wrong patient 10, and other 5. The hospital recently upgraded its infusion pumps. Using Pareto analysis, which categories should be targeted first?

- A. All five categories equally
- B. Wrong patient, because it seems most dangerous
- C. Wrong dose and wrong time, which together make up 70% of errors
- D. Wrong dose only

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7. [TRANSFER — reversed stem / different variable]

In the same data (45, 25, 15, 10, 5 of 100 errors), what cumulative percentage do the top three categories account for?

- A.** 15%
- B.** 95%
- C.** 85%
- D.** 70%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

After a wrong-site surgery at a hospital that recently won a teaching award, the RCA team finds the time-out was rushed because of pressure to meet OR turnover targets, and the site-marking policy is unclear. The surgeon is well known in the community. What should the corrective action focus on?

- A.** Discipline the surgeon
- B.** Clarify the marking policy, protect the time-out, and review turnover pressure
- C.** Retrain only the staff in the room
- D.** Nothing, since wrong-site surgery can never be prevented

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A nurse on a unit using barcode medication scanning skips scanning because the scanner is slow and other nurses do the same. No harm occurred. She was named nurse of the year last spring. Under Just Culture, how should this be handled?

- A.** Coach her on the at-risk behavior and fix the slow scanner that encourages the shortcut
- B.** Console her, since it was an honest mistake
- C.** Take no action, because no harm occurred
- D.** Discipline her for reckless behavior

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

A pharmacist knowingly and repeatedly overrides dose-limit warnings, fully aware of the danger. Under Just Culture, what response fits?

- A.** Consoling for human error
- B.** No action unless a patient is harmed
- C.** Disciplinary action for reckless behavior
- D.** Coaching for at-risk behavior

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A hospital with a highly rated cafeteria launches a project to shorten length of stay. The project lead holds a Six Sigma black belt. Which balancing measure should the team track?

- A.** 30-day readmission rate
- B.** None, since a lower length of stay always means success
- C.** Percentage of discharges before noon
- D.** Average length of stay

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A hospital in a city with a growing tech industry finds that Spanish-speaking patients wait longer for pain medication than English-speaking patients with the same pain scores. The hospital recently added valet parking. Which quality aim is most at stake?

- A.** Efficient
- B.** Safe, because every quality problem is a safety problem
- C.** Timely
- D.** Equitable

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A hospital ~~recently featured in a regional magazine~~ tracks the percentage of surgical patients who get antibiotics within 60 minutes before incision. ~~Its surgical wing has 12 operating rooms.~~ In Donabedian's model, this is which type of measure?

Target word: measure type **Decides:** It measures an action done for patients.

A	Absolute language	Percentages can describe processes, structures, or outcomes.
B	Correct	Giving antibiotics on time is a care process.
C	Camouflage	The room count is structure, but it is not what is being measured.
D	Confusion set	The infection rate would be the outcome; this measures the action.

2. Answer A Concept 1 • transfer

For the same surgical program, which of these is an outcome measure?

Target word: outcome example **Decides:** A result in patients' health.

A	Correct	Infection is a patient health result.
B	Adjacent category	That is a structure measure.
C	Confusion set	That is a process measure.
D	Absolute language	Public reporting does not make a measure an outcome.

3. Answer D Concept 2

A clinic ~~in a town with a new minor league stadium~~ wants to cut no-shows. Staff predict that text reminders sent two days ahead will lower no-shows, and they try it with one provider for two weeks. ~~The clinic manager just finished her MBA.~~ What happens in the Study step?

Target word: Study step **Decides:** Compare results with the prediction.

A	Inverted operation	Reverses PDSA's small, rapid testing approach.
B	Confusion set	That is the Do step.
C	Adjacent category	Adopting or spreading is the Act step.
D	Correct	Study analyzes data against the prediction.

4. Answer B Concept 3

A lab turnaround-time control chart at a hospital ~~that recently renovated its lab~~ shows daily values varying within the control limits with no pattern. On Tuesday, one value is higher than Monday's but still well within the limits. ~~The lab director is a former Army medic.~~ What should the team do?

Target word: response to the point **Decides:** Within limits, no pattern: common cause.

A	Confusion set	Nothing signals special cause; the point is within limits.
B	Correct	Reacting to common-cause noise (tampering) adds variation.
C	Adjacent category	Limits are not reset for normal variation.
D	Absolute language	Rises within limits are expected in a stable process.

5. Answer D Concept 3 · transfer

On a control chart, which pattern signals special-cause variation?

Target word: special-cause signal **Decides:** A point outside limits or a sustained run.

A	Dropped element	Drops the run rule; a signal needs a long run on one side, not one point.
B	Confusion set	That is ordinary common-cause variation.
C	Inverted operation	That is the picture of a stable process.
D	Correct	Points beyond the limits signal special cause.

6. Answer C Concept 4

A medication-error review at a hospital ~~with a new pharmacy director~~ found 100 errors: wrong dose 45, wrong time 25, omitted dose 15, wrong patient 10, and other 5. ~~The hospital recently upgraded its infusion pumps.~~ Using Pareto analysis, which categories should be targeted first?

Target word: vital few **Decides:** Top categories by frequency.

A	Absolute language	Pareto focuses on the vital few.
B	Adjacent category	Severity ranking is a different tool; Pareto ranks by frequency.
C	Correct	$45 + 25 = 70$ of 100.
D	Dropped element	Drops wrong time, the second-largest category.

7. Answer C Concept 4 · transfer

In the same data (45, 25, 15, 10, 5 of 100 errors), what cumulative percentage do the top three categories account for?

Target word: cumulative percent **Decides:** $45 + 25 + 15$.

A	Dropped element	That is the third category alone, not the running total.
B	Confusion set	That is the top four categories.
C	Correct	$45 + 25 + 15 = 85$ of 100.
D	Wrong stem number	That is the top two categories only.

8. Answer B Concept 5

After a wrong-site surgery at a hospital ~~that recently won a teaching award~~, the RCA team finds the time-out was rushed because of pressure to meet OR turnover targets, and the site-marking policy is unclear. ~~The surgeon is well known in the community.~~ What should the corrective action focus on?

Target word: focus of action **Decides:** Findings point to system conditions.

A	Adjacent category	Blame does not fix the unclear policy or time pressure.
B	Correct	RCA fixes the system conditions that allowed the error.
C	Dropped element	Drops the policy and workflow causes the RCA found.
D	Absolute language	It is considered preventable with reliable systems.

9. Answer A Concept 6

A nurse on a unit using barcode medication scanning skips scanning because the scanner is slow and other nurses do the same. No harm occurred. ~~She was named nurse of the year last spring.~~ Under Just Culture, how should this be handled?

Target word: Just Culture response **Decides:** A drift from safe practice where risk is not seen.

A	Correct	At-risk behavior calls for coaching plus removing system incentives.
B	Confusion set	Human error is unintended; skipping scanning was a choice.
C	Dropped element	Drops the behavior; Just Culture judges behavior, not outcome.
D	Inverted operation	Reckless means consciously disregarding a known substantial risk.

10. Answer C Concept 6 · transfer

A pharmacist knowingly and repeatedly overrides dose-limit warnings, fully aware of the danger. Under Just Culture, what response fits?

Target word: Just Culture response **Decides:** Conscious disregard of known risk.

A	Adjacent category	Human error is unintended.
B	Dropped element	Drops the behavior test; outcome does not decide.
C	Correct	Conscious disregard of substantial risk is reckless.
D	Confusion set	At-risk behavior involves risk the person does not recognize.

11. Answer A Concept 7

A hospital ~~with a highly rated cafeteria~~ launches a project to shorten length of stay. ~~The project lead holds a Six Sigma black belt.~~ Which balancing measure should the team track?

Target word: balancing measure **Decides:** Watch for harm from discharging too soon.

A	Correct	Readmissions could rise if patients leave too early.
B	Absolute language	Improvement in one area can cause harm elsewhere.
C	Adjacent category	That is a process measure driving the change.
D	Confusion set	That is the project's outcome measure.

12. Answer D Concept 8

A hospital ~~in a city with a growing tech industry~~ finds that Spanish-speaking patients wait longer for pain medication than English-speaking patients with the same pain scores. ~~The hospital recently added valet parking.~~ Which quality aim is most at stake?

Target word: quality aim **Decides:** Care differs by a personal characteristic.

A	Adjacent category	Efficiency concerns waste, not differences between groups.
B	Absolute language	The aims are distinct, and equity fits best here.
C	Confusion set	Delay is the symptom; the gap between groups is the issue.
D	Correct	Care that varies by language for the same need is an equity gap.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	6	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	8	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	3	
Absolute language	Always, never, guarantee — where judgment applies.	7	
Confusion set	Every option is real; the test is exact role-matching.	10	
Camouflage	Answers a stem fact that was never the question.	1	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Donabedian structure-process-outcome	Structure is the setting and resources, process is what is done for patients, and outcome is the result for patients' health.	Calling a care-delivery percentage an outcome because it is a number.	Whether the measure counts resources, actions, or results.
2	PDSA cycle	Plan a small test with a prediction, Do it, Study results against the prediction, then Act to adopt, adapt, or abandon.	Skipping the prediction or launching full-scale before small tests.	A small test with one provider or one unit.
3	Common vs. special cause variation	Points within control limits with no pattern are common-cause variation; react to special-cause signals such as a point outside the limits or a sustained run.	Investigating every up-tick as if it were a special event.	A single higher value still within limits.
4	Pareto analysis	Rank causes by frequency and target the vital few categories that account for most of the problem, using cumulative percentages.	Spreading effort evenly or starting with the rarest category.	A frequency table of error types.
5	Root cause analysis and systems thinking	RCA looks past individual error to the system conditions that allowed it and fixes those conditions.	Ending the analysis with blame or retraining one person.	An adverse event with policy or workflow problems in the findings.
6	Just Culture	Console human error, coach at-risk behavior (a drift from safe practice where risk is not seen) and fix system incentives, and discipline reckless behavior; judge the behavior, not the outcome.	Letting the outcome, not the behavior, decide the response.	A shortcut "everyone takes" versus a conscious choice to disregard risk.
7	Balancing measures	Track a balancing measure to catch unintended harm elsewhere in the system when improving one area.	Assuming improvement in the target measure proves overall success.	A project pushing one measure hard, such as shorter length of stay.
8	The six aims for quality (STEEEP)	Care should be safe, timely, effective, efficient, equitable, and patient-centered; equity means quality does not vary by personal characteristics.	Labeling a gap between groups by the symptom (delay) instead of the disparity.	A difference in care between groups with the same clinical need.

Course scope (public description): The Healthcare Quality course equips learners with the knowledge and skills necessary to evaluate, measure, and enhance the quality of healthcare services. By combining theoretical foundations with practical applications, the course prepares learners to lead impactful quality improvement initiatives, analyze healthcare outcomes, and implement strategies that promote patient safety, satisfaction, and overall care excellence. Through the use of performance metrics, learners will design a comprehensive quality improvement project tailored to a healthcare organization's quality improvement plan. This course fosters critical thinking and leadership, enabling participants to drive meaningful change in healthcare quality and patient outcomes. Prerequisite: There are no prerequisites to the course.